L48 - AAROGYAMM PATHOLOGY
S. NO. 35, SHOP NO 14, GROUND FLOOR ARC
INFRONT OF RENUKA MATA MANDIR

NABL ACCREDITED
MC-2762

Name	: **Mr. R.S. MITTAL**			**Collected**	: **24/2/2019 12:05:00PM**
Lab No.	: **264184526**	**Age: 58 Years**	**Gender: Male**	**Received**	: **24/2/2019 12:07:50PM**
				Reported	: **25/2/2019 11:33:57AM**
A/c Status	: **P**	**Ref By : Dr. GAWARE**		**Report Status**	: **Regular**

Test Name	Results	Units	Bio. Ref. Interval
LIPID PROFILE, SCREEN (Enzymatic)			
Cholesterol, Total	136.90	mg/dL	<200.00
Triglycerides	**176.06**	mg/dL	<150.00
HDL Cholesterol	**9.10**	mg/dL	>40.00
LDL Cholesterol, Calculated	92.59	mg/dL	<100.00
VLDL Cholesterol,Calculated	**35.21**	mg/dL	<30.00
Non-HDL Cholesterol	128	mg/dL	<130

Result Rechecked on fresh sample,
Please Correlate Clinically.

Interpretation

REMARKS	TOTAL CHOLESTEROL in mg/dL	TRIGLYCERIDE in mg/dL	LDL CHOLESTEROL in mg/dL	NON HDL CHOLESTEROL in mg/dL
Optimal	<200	<150	<100	<130
Above Optimal	-	-	100-129	130 - 159
Borderline High	200-239	150-199	130-159	160 - 189
High	>=240	200-499	160-189	190 - 219
Very High	-	>=500	>=190	>=220

Note

1. Measurements in the same patient can show physiological & analytical variations. Three serial samples 1 week apart are recommended for Total Cholesterol, Triglycerides, HDL& LDL Cholesterol.
2. NLA-2014 recommends a complete lipoprotein profile as the initial test for evaluating cholesterol.
3. Friedewald equation to calculate LDL cholesterol is most accurate when Triglyceride level is < 400 mg/dL. Measurement of Direct LDL cholesterol is recommended when Triglyceride level is > 400

If test results are alalrming or unexpected, client is advised to contact the laboratory immediately for possible remedial action.
@ Tests conducted at Referral Lab.

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mg/dL

4. NLA-2014 identifies Non HDL Cholesterol(an indicator of all atherogeniclipoproteins such as LDL , VLDL, IDL, Lpa, Chylomicron remnants)along with LDL-cholesterol as co- primary target for cholesterol lowering therapy. Note that major risk factors can modify treatment goals for LDL &Non HDL.
5. Apolipoprotein B is an optional, secondary lipid target for treatment once LDL & Non HDL goals have been achieved
6. Additional testing for Apolipoprotein B, hsCRP,Lp(a) & LP-PLA2 should be considered among patients with moderate risk for ASCVD for risk refinement

Treatment Goals as per Lipid Association of India 2016

RISK CATEGORY	TREATMENT GOAL		CONSIDER THERAPY	
	LDL CHOLESTEROL (LDL-C)(mg/dL)	NON HDL CHLOESTEROL (NON HDL-C) (mg/dL)	LDL CHOLESTEROL (LDL-C)(mg/dL)	NON HDL CHLOESTEROL (NON HDL-C) (mg/dL)
Very High	<50	<80	>=50	>=80
High	<70	<100	>=70	>=100
Moderate	<100	<130	>=100	>=130
Low	<100	<130	>=130*	>=160*

*In low risk patient, consider therapy after an initial non-pharmacological intervention for at least 3 months

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IMPORTANT INSTRUCTIONS

*Test results released pertain to the specimen submitted.*All test results are dependent on the quality of the sample received by the Laboratory. *Laboratory investigations are only a tool to facilitate in arriving at a diagnosis and should be clinically correlated by the Referring Physician .*Sample repeats are accepted on request of Referring Physician within 7 days post reporting.*Report delivery may be delayed due to unforeseen circumstances. Inconvenience is regretted.*Certain tests may require further testing at additional cost for derivation of exact value. Kindly submit request within 72 hours post reporting.*Test results may show interlaboratory variations.*The Courts/Forum at Delhi shall have exclusive jurisdiction in all disputes/claims concerning the test(s) & or results of test(s).*Test results are not valid for medico legal purposes. *Contact customer care Tel No. +91-11-39885050 for all queries related to test results.

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